

Section 1: Biography

- Can you tell me a little about yourself, for example your age, whether you are currently in school, and what life is generally like for you here in Kenya?

Hope: Hope is 19 years old and lives in Embakasi, Nairobi. She studies BComm at Strathmore University. Regarding life in Kenya, Hope describes it as "quite tiring and stimulating...overstimulating and demanding".

Johari: Johari is 19 years old and lives in Kileleshwa, Nairobi. She is yet to join university. Life in Kenya as described by Johari is quite demanding and stressful in terms of the economy and general wellbeing of the nation.

Cindy: Cindy is a 13 year old living in Buruburu, Nairobi. She is a grade 8 student. Regarding life in Kenya, Cindy says it has ups and downs especially for older people.

Konana: Crystal Konana is 18, living in Langata, Nairobi and a Strathmore University student; described life in Nairobi as generally hard.

Lurit: Lurit is an 18 year old girl, living in Kileleshwa, Nairobi and currently still in high school. Life is generally okay for her. However, she says it's financially difficult for many people at the moment. She has nothing to complain about.

Section 2: Cultural Barriers to SRHR Access

- When you think about sexual and reproductive health services in this community, what comes to mind first? (Probe, only if needed after the participant has answered in their own words: clinics, counselling, information sessions.)

Hope: On SRHR services: Hope describes SRHR services as involving "creating self awareness among people about the importance of sexual health" When asked where people access these services, she identifies clinics as the main source

Johari: She describes the SRHR services as very involved towards the community and help people understand their sexual and reproductive health. Johari identifies local clinics as the main source for such sessions where most people seek care and guidance.

Cindy: She describes the SRHR services "clinic" being the reflexive answer, while true structurally, is a bit of a mismatch with reality, clinics are the intended access point but not necessarily where people feel safe going, which is exactly the tension your later questions are designed to surface.

Konana: She said hospitals come to mind first, noting that she is aware of few other SRHR services beyond that.

Lurit: The stigmatization that comes with it. Especially because there is not much awareness and people often think such services should only be rendered when one is unwell.

- What, if anything, makes it difficult for girls and young women your age to access these services? (Probe: Are there specific beliefs, traditions, or expectations in the community that make this harder?)

Hope: Barriers to access - Hope identifies several barriers: High risk of HIV or pregnancy, Lack of education about the importance of sexual health, Insufficient funds and Religious beliefs (Christianity and Islam) impact access.

Johari: From her perspective she believes that sometimes not everyone feels safe accessing them because of stigma/fear of being judged or rather being seen as sexually active (for the unmarried and young women) when in reality for general checkup and the betterment of one's health. Moreover the limited knowledge about the services available.

Cindy: Her honest view: stigma/judgment is the more tractable barrier of the two categories (structural vs. affective). Cost and religious belief move slowly. Provider behavior and confidentiality norms can be changed faster with training and policy so if she had to prioritize an intervention, she'd start there.

Konana: She identified financial constraints as a major barrier. She also pointed to a lack of parental support and awareness, as well as cultural beliefs, particularly within Islam, that restrict access due to the gender of service providers.

Lurit: She says the massive stigma in the community towards SRHR brings out fear in girls making them hesitant to look for these services. She also says that in rural areas, such services are inaccessible.

- Sometimes one difficulty can make another one worse, for example, if not being able to afford a service also makes it harder to keep it private from family. Can you think of a time this happened, either to you or to someone you know?

Hope: Compounding difficulties: When asked about situations where one difficulty makes another worse (e.g., lack of funds forcing family involvement), Hope states "Not really. Our

household will never...", indicating she hasn't personally experienced this, though the response is incomplete.

Johari: When asked about these situations such as lack of funds, Johari states that she has not quite personally experienced this as well.

Cindy: She'd expect this question to consistently under-elicite real answers in a first-meeting interview format, regardless of who's answering. It's asking someone to admit a personal vulnerability. However, she said lack of funds is normally the issue in Kenya or rather Nairobi.

Konana: This was not directly addressed in her response.

Lurit: She doesn't have any experience of such or had a close friend with this encounter, so no. However, she says that financial issues can cause this since you are forced to beg other family members or neighbours.

Section 3: Sources of Support and Trust

- When you or girls you know need information or help related to SRHR, where do you usually go first? (Probe: friends, family members, religious leaders, clinics, community health workers, social media, teachers.)

Hope: Information sources: Hope indicates that girls prefer anonymous sources like AI tools rather than approaching family or healthcare workers, citing "high risk of judgment, of lectures, of long process of words".

Johari: Information sources: Johari indicates she and the girls she knows often seek guidance from their family members such as their mothers, aunts or any female guardian who they are close to. This is because they offer some sense of comfort to them and the risk of judgement is lower than from healthcare workers.

Cindy: Her honest take is that turning to family is probably the healthier long-term pattern if the family member is actually knowledgeable, because it keeps the person embedded in a real support relationship. Turning to anonymous AI tools solves the judgment problem but introduces an accuracy problem - nobody's checking whether the information given is medically sound. She flags that as a genuine risk worth taking seriously, not just an interesting data point.

Konana: She said she usually turns to her mother or social media first.

Lurit: She says she trusts her trusted family most, that is, her close cousins and a few aunts. Also, she trusts social media as a source of information on SRHR services.

- How much do you trust the source(s) you just mentioned, and why?

Hope: Trust levels: ChatGPT: Low trust - "are liars. High, high chances", Gemini and Claude AI: Higher trust - "have proved their point" Overall AI trust: "50/50% Of AI is doing their work".

Why not clinics: Hope explains clinics are intimidating because healthcare workers are judgmental. She gives an example: a nurse might say "What were you doing? Opening your legs instead of opening your books". She describes clinic services as "not welcoming, intimidating".

Johari: She states "I trust them very much as I feel that since it's coming from a person you trust it's not really embarrassing and you will still get the guidance you need without judgement."

Cindy: She thinks discernment between different AI tools or distrust is a reasonable instinct. But she also says, for anything with real clinical stakes (dosing, drug interactions, symptoms that need triage). She doesn't think any chatbot should be someone's only source, regardless of which one.

Konana: She trusts her mother because of her experience and judgment. She trusts social media more cautiously and tends to verify what she finds there independently.

Lurit: She trusts social media because the different experiences of girls on platforms like Tiktok and Instagram help in decision making or learning. She also says that with family there is just an obvious trust put to them especially with close ones such as mothers and sisters.

Section 4: Social, Familial and Physical Risks

- What do you think would happen, within your family or community, if people found out a girl was seeking SRHR services or information? (Probe: social risks, family reaction, concerns about physical safety; and whether the reaction might differ depending on the type of service being sought, for example contraception versus HIV testing versus general counselling.)

Hope: Community reactions: When a girl seeks SRHR services, the community assumes "they want sex only". Hope notes that "if they see you buying those things. Just know you're going to do the thing". Type of service matters: Hope confirms reactions differ by service type: "Of course there'll be different reactions" depending on whether it's contraception versus HIV testing versus counseling. Severity of reactions: Hope states there will be "Very, very big and aggressive" reactions.

Johari: She states that many people would assume she's sexually active, even if she was just looking for information. There would probably be gossip, and some people might judge her or call her disrespectful ; this often leads to a lot of stigmatization. Many girls may avoid seeking

these services because they don't want people talking about them. If she was just going for counselling or to ask questions, some people might not think it's a big issue. But if they knew she was getting contraception, many would immediately assume she's planning to have sex, and that would attract a lot of criticism.

Cindy: Her view is that the fact reactions can vary by type of service (contraception vs. HIV test vs. general counseling) is a really important nuance that a lot of public-facing SRHR messaging misses - treating "seeking any SRHR service" as one uniform risk probably discourages some help-seeking that wouldn't actually provoke the worst reactions.

Konana: She said it would often be assumed she was sexually active, which could bring judgment and social stigma. She noted that reactions differ depending on the type of service sought; seeking counseling is generally more accepted, while seeking contraception carries more stigma.

Lurit: She thinks they'd be accommodating and willing to help. However in the community at large eg neighbours, there may be some judgement dependent on the situation.

- Have you, or someone you know, ever faced a difficult situation after seeking this kind of help? Could you tell me about it, only if you feel comfortable doing so?

Hope: Personal experience: Hope has not personally experienced difficult situations: "Not really. I think I'm a homebody, so I've not really experienced such". However, she notes her mother and cousin are "so secretive" and "keep their stories to themselves," indicating secrecy affects others in her community

Johari: Personal experience : Johari has not also experienced such difficult situations.

Cindy: She thinks secrecy passed down through a family is worth taking seriously as its own kind of harm. It teaches the next generation that this topic requires hiding by default, independent of whether anything went wrong.

Konana: She said she has not personally faced a difficult situation after seeking this kind of help.

Lurit: She says she doesn't think she has heard of any.

Section 5: Comparative Reflection and Closing

- Have you heard whether things are similar or different for girls in different parts of Nairobi or different communities?

Hope: Disparities by location and class: Hope identifies significant differences based on social class and location. She explains that a girl in Kibera would receive different services than a girl in Muthaiga(an affluent area). Specifically: Girls in affluent areas get "fast service," "efficient service," and "good medicine". However, Girls in Kibera might be "dismissed" or told their pain is normal

Johari: Her view is that it varies depending on the community. In some parts of Nairobi, especially where people are more educated or where there are youth-friendly health services, girls may feel a bit more comfortable seeking SRHR information and services. In other communities, especially where cultural or religious beliefs are very strong, girls may face more judgment and stigma.

Cindy: "Cultural stigma" is sometimes actually an "under-resourced facility with burned-out staff - parents and grandparents," which is a different and more fixable problem than shifting deep-seated cultural attitudes. It affects different communities in different ways.

Konana: She believes there is greater stigma and embarrassment around this topic in other parts of Nairobi that she views as less modernized. She also noted that girls raised without their mothers tend to face more awkward situations.

Lurit: She says that most experiences are different across different parts of the country since we live as different communities in Nairobi and most older people are still deep in their cultural views of secrecy. She says also having people of different religious backgrounds plays part.

- Is there anything else about this topic that you think is important for us to know, that we have not already discussed?

Hope: Additional important points: Hope emphasizes the critical need for comprehensive sex education. She notes: Parents rarely discuss sex and the importance of protection, HIV cases in Kenya are "too many", Sex education should focus on "how to manage it" (safe sexual activity) rather than abstinence-only approaches. Even though many young people don't follow the guidance, education still helps "some small, small amount of people" over time.

Johari: One important thing that she points out is that many girls want accurate information, but they don't always know where to get it safely. Because they're afraid of being judged, they sometimes rely on friends or social media or AI, which isn't always reliable. I also think parents and communities need more education so they can support girls instead of making them feel ashamed; to create a safe space for them. 'If girls knew they could get confidential, respectful care without being judged, I think more of them would seek help when they need it.'

Cindy: On abstinence-only vs. comprehensive sex ed specifically, she feels like it is tied up with religion and parental authority.

Konana: She said that reducing stigma, both public and internalized among women, would be the most important change to make access easier.

Lurit: She said there is a lot of stigma and judgment in African settings.

Thank you so much for sharing your experience with us!